

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 07	Duration: 3 hours
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 8/20	Training reference: VTI-AEDA examination style

COMPREHENSIVE VOCATIONAL PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 07

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

How to use this marking guide

For revision, first attempt the paper without notes. Then compare not only the answer letter but the clinical cue that makes the answer correct. For legal/ethical items, the guide follows the concepts emphasized by the supplied course/examination materials; in real practice, applicable Cameroon law, institutional policy and the current professional code must also be checked.

PART ONE: PSYCHOPATHOLOGY - ANSWERS

1. Wendy, a 60-year-old mechanic, presents with significantly low body weight, intense fear of weight gain and restriction without recurrent binge-purge episodes. Which diagnosis or clinical concept best fits this presentation?

- A. Major depressive disorder
- B. Autism spectrum disorder
- C. Panic disorder

D. Anorexia nervosa, restricting type

Answer: D - Anorexia nervosa, restricting type. Anorexia nervosa requires significantly low weight plus fear of gaining weight or persistent behavior interfering with weight gain and body-image disturbance.

Memory aid: ANOREXIA: restriction + low weight + fear of weight gain.

2. During assessment, Amina reports significantly low body weight with anorexia features plus recurrent binge-eating or purging. Which of the following is the BEST match?

- A. Social anxiety disorder
- B. Schizoid personality disorder

C. Anorexia nervosa, binge-eating/purging type

- D. Brief psychotic disorder

Answer: C - Anorexia nervosa, binge-eating/purging type. The patient meets anorexia criteria including low weight, but also has recurrent binge/purge behavior.

Memory aid: Low weight first: if anorexia criteria are met, binge/purge does not automatically mean bulimia.

3. Carlos, a 36-year-old laboratory technician, presents with recurrent binge eating followed by compensatory behaviors such as vomiting, laxatives or excessive exercise, without the persistently low weight required for anorexia. Which diagnosis or clinical concept best fits this presentation?

- A. Substance use disorder
- B. Obsessive-compulsive personality disorder (OCPD)
- C. Anorexia nervosa, restricting type

D. Bulimia nervosa

Answer: D - Bulimia nervosa. Bulimia is defined by binge episodes and inappropriate compensation; weight may be normal or above/below normal but not at the markedly low anorexia threshold.

Memory aid: BULIMIA = Binge + Undo.

4. A therapist notes that Grace has recurrent binge eating with loss of control and distress but without regular compensatory purging or fasting. Which condition should be considered first from the options below?

- A. Schizophrenia
- B. Bipolar disorder
- C. Binge-eating disorder**
- D. Schizoid personality disorder

Answer: C - Binge-eating disorder. Binge-eating disorder differs from bulimia because recurrent compensatory behaviors are absent.

Memory aid: *BED = Binge, no regular Erasing behavior.*

5. Victor, a 23-year-old teacher, presents with a problematic pattern of substance use causing clinically significant impairment or distress, including loss of control, risky use, social impairment or tolerance/withdrawal. Which diagnosis or clinical concept best fits this presentation?

- A. Anorexia nervosa, binge-eating/purging type
- B. Autism spectrum disorder
- C. Substance use disorder**
- D. Generalized anxiety disorder (GAD)

Answer: C - Substance use disorder. Substance use disorder is diagnosed from a maladaptive pattern and consequences, not simply from having used a substance.

Memory aid: *4C idea: Control loss, Consequences, Compulsion/craving, Continued use.*

6. A therapist notes that Carlos has initial euphoria followed by drowsiness or apathy, with slowed functioning and often constricted pupils. Which condition should be considered first from the options below?

- A. Specific phobia
- B. Opioid intoxication**
- C. Post-traumatic stress disorder (PTSD)
- D. Anorexia nervosa, restricting type

Answer: B - Opioid intoxication. Opioids typically produce analgesia, euphoria followed by sedation and physiologic slowing.

Memory aid: *OPIOID = Often Pinpoint pupils, Inward/sedated, Opioid drowsiness.*

7. During assessment, Tina reports euphoria, increased energy and confidence, decreased need for sleep, agitation and autonomic activation. Which of the following is the BEST match?

- A. Brief psychotic disorder
- B. Stimulant intoxication**
- C. Dependent personality disorder
- D. Post-traumatic stress disorder (PTSD)

Answer: B - Stimulant intoxication. Stimulants activate rather than sedate; cocaine and amphetamines commonly produce energy, alertness and agitation.

Memory aid: *STIMULANT = STIMulates.*

8. During assessment, Nadine reports pervasive instability in relationships, self-image and affect, impulsivity, abandonment fears and recurrent self-harm or suicidal behavior. Which of the following is the BEST match?

- A. Borderline personality disorder**
- B. Generalized anxiety disorder (GAD)
- C. Schizophrenia
- D. Obsessive-compulsive personality disorder (OCPD)

Answer: A - Borderline personality disorder. Borderline personality disorder is marked by instability and intense reactivity across relationships, identity, mood and behavior.

Memory aid: *BORDER: Bonds unstable, Overreactive affect, Risky acts, Desertion fear, Empty/identity shifts, Recurrent self-harm.*

9. During assessment, Yvette reports perfectionism, orderliness, rigidity and control that interfere with flexibility and task completion. Which of the following is the BEST match?

- A. Obsessive-compulsive personality disorder (OCPD)**
- B. Agoraphobia

- C. Schizoid personality disorder
- D. Dependent personality disorder

Answer: A - Obsessive-compulsive personality disorder (OCPD). OCPD is a personality pattern of perfectionism and control; unlike OCD, true obsessions and compulsions are not required.

Memory aid: OCPD = Order, Control, Perfectionism, Duty.

10. A therapist notes that Daniel has detachment from social relationships and restricted emotional expression with little desire for close relationships. Which condition should be considered first from the options below?

- A. Schizoid personality disorder
- B. Obsessive-compulsive disorder (OCD)
- C. Manic episode
- D. Obsessive-compulsive personality disorder (OCPD)

Answer: A - Schizoid personality disorder. The key pattern is social detachment and limited affect, rather than fear of rejection as in avoidant personality.

Memory aid: SCHIZOID = Solitary by preference.

SECTION B: TRUE/FALSE - EXPLANATIONS

1. FALSE. Schizoid personality disorder is defined primarily by intense fear of criticism and rejection.

Schizoid personality is primarily social detachment with limited desire for close relationships.

2. TRUE. Autism spectrum disorder includes both social-communication difficulties and restricted/repetitive behavior patterns.

Both domains are required in the modern diagnostic framework.

3. TRUE. The four D framework can include deviance, distress, dysfunction and danger.

These four dimensions are commonly taught as aids for thinking about abnormality.

4. TRUE. Comorbidity means that two or more disorders can occur in the same person.

Comorbidity is the co-occurrence of disorders.

5. FALSE. A specific phobia is generalized worry across many areas of life.

Generalized worry across many domains is more characteristic of GAD.

SECTION C: MODEL SHORT ANSWERS

1. Differentiate a panic attack from panic disorder. (2mks)

- A panic attack is an acute episode of intense fear/discomfort with physical/cognitive symptoms.
- Panic disorder requires recurrent unexpected attacks plus persistent concern and/or maladaptive behavior change.

Memory aid: Attack = episode; disorder = recurring pattern + aftermath.

2. Differentiate obsession from compulsion and give one example of each. (2mks)

- Obsession: recurrent intrusive unwanted thought/urge/image, e.g. fear of contamination.
- Compulsion: repetitive behavior/mental act to reduce distress, e.g. repeated hand washing.

Memory aid: O = thought; C = conduct.

3. State the four D criteria commonly used when considering abnormality. (2mks)

- Deviance
- Distress
- Dysfunction
- Danger

Memory aid: 4D: Deviance, Distress, Dysfunction, Danger.

4. Define substance use disorder in practical clinical terms. (2mks)

- Problematic pattern of substance use causing significant impairment or distress.
- Look for loss of control, social/role impairment, risky use and/or tolerance/withdrawal.

Memory aid: 4C: Control loss, Consequences, Craving, Continued use.

5. Give two reasons early intervention in substance use disorder is important. **(2mks)**

- Reduces medical, social, occupational and legal harm and risk of escalation.
- Improves opportunity for engagement, relapse prevention, family support and recovery before consequences accumulate.

Memory aid: *EARLY = less Entrenchment, Address Risk, Link help, Yield recovery.*

PART TWO: THEORIES/TECHNIQUES - ANSWERS

1. Which psychotherapy concept is most accurately described by the following: the client redirects feelings, expectations or relationship patterns from important past relationships onto the therapist?

- A. Flooding
- B. Empty-chair technique
- C. Transference**
- D. Gestalt therapy

Answer: C - Transference. Transference originates primarily from the client's earlier relationships and becomes enacted in the therapeutic relationship.

Memory aid: TRANSference = client TRANSfers old relationship feelings to therapist.

2. In a session with Irene, the counselor emphasizes the therapist's emotional reactions to a client, especially reactions influenced by the therapist's own history or unresolved issues. This BEST illustrates:

- A. Resistance
- B. Transference
- C. Negative reinforcement
- D. Countertransference**

Answer: D - Countertransference. Countertransference concerns the therapist's responses to the client and must be monitored through self-awareness and supervision.

Memory aid: COUNTER = counselor side of the relational reaction.

3. Which psychotherapy concept is most accurately described by the following: therapy emphasizing unconscious conflict, early relationships, defenses, transference, resistance and insight?

- A. Transference
- B. Gestalt therapy
- C. Psychoanalytic therapy**
- D. Progressive muscle relaxation

Answer: C - Psychoanalytic therapy. Psychoanalytic approaches seek insight into unconscious patterns and how early relationships repeat in present life and therapy.

Memory aid: FIRT: Free association, Interpretation, Resistance, Transference.

4. Which psychotherapy concept is most accurately described by the following: a goal-directed, socially embedded approach emphasizing conscious choice, responsibility, lifestyle and movement from inferiority toward useful belonging?

- A. Adlerian therapy**
- B. Social skills training
- C. Cognitive restructuring
- D. Extinction

Answer: A - Adlerian therapy. Adlerian therapy views behavior as purposeful and socially related, with emphasis on goals, choice and responsibility. This is explicitly reflected in the opening lecture of the fused audio.

Memory aid: GSCR: Goal-directed, Social relatedness, Conscious choice, Responsibility.

5. Which psychotherapy concept is most accurately described by the following: a non-directive relationship based on accurate empathy, unconditional positive regard and congruence?

- A. In vivo exposure
- B. Free association
- C. Countertransference
- D. Person-centered therapy**

Answer: D - Person-centered therapy. Rogers' person-centered therapy relies on core facilitative conditions rather than therapist-imposed solutions.

Memory aid: EUC: Empathy, Unconditional positive regard, Congruence.

6. A therapist working with Lucas uses an intervention characterized by a structured approach linking thoughts, emotions and behaviors and using cognitive and behavioral change strategies. Which approach or technique is being used?

- A. Gestalt therapy
- B. Extinction
- C. Resistance

D. Cognitive behavioral therapy (CBT)

Answer: D - Cognitive behavioral therapy (CBT). CBT targets maladaptive thinking and behavior through collaborative, structured methods such as cognitive restructuring and behavioral experiments.

Memory aid: TEA: Thoughts - Emotions - Actions.

7. In a session with Zachary, the counselor emphasizes identifying inaccurate or dysfunctional thoughts, examining evidence and developing more balanced alternatives. This BEST illustrates:

- A. Gestalt dream work
- B. Cognitive restructuring**
- C. Social modeling
- D. Systematic desensitization

Answer: B - Cognitive restructuring. Cognitive restructuring is a core CBT method for testing and modifying distorted or unhelpful cognitions.

Memory aid: Catch it, Check it, Change it.

8. Which psychotherapy concept is most accurately described by the following: Albert Ellis' approach that identifies irrational beliefs between events and emotional consequences and actively disputes those beliefs?

- A. Progressive muscle relaxation
- B. Social skills training
- C. Resistance

D. Rational Emotive Behavior Therapy (REBT)

Answer: D - Rational Emotive Behavior Therapy (REBT). REBT uses the ABC model and disputation to replace rigid irrational beliefs with more rational alternatives.

Memory aid: ABCDE: Activating event, Belief, Consequence, Dispute, Effective new belief.

9. Which psychotherapy concept is most accurately described by the following: an REBT imagery exercise in which the client vividly imagines a difficult event, experiences the emotion, then practices changing unhealthy emotion by changing beliefs?

- A. Rational emotive imagery**
- B. Cognitive restructuring
- C. Flooding
- D. Free association

Answer: A - Rational emotive imagery. Rational emotive imagery is an experiential REBT technique for rehearsing healthier emotional responses through belief change.

Memory aid: Imagine - feel - dispute - re-feel.

10. A therapist working with Hassan uses an intervention characterized by a here-and-now experiential approach emphasizing awareness, contact, unfinished business and integration. Which approach or technique is being used?

- A. Social modeling
- B. Gestalt therapy**
- C. Free association
- D. Adlerian therapy

Answer: B - Gestalt therapy. Gestalt therapy focuses on present awareness and experiments that help clients own and integrate experience.

Memory aid: Gestalt = awareness in the NOW.

11. A therapist working with Uma uses an intervention characterized by the client speaks to an imagined person or part of self placed in an empty chair, often switching roles to deepen awareness. Which approach or technique is being used?

- A. Progressive muscle relaxation
- B. Operant conditioning
- C. Empty-chair technique**
- D. Countertransference

Answer: C - Empty-chair technique. The empty-chair experiment is associated with Gestalt work on unfinished business, polarities and present-moment experience.

Memory aid: *Empty chair = make the absent relationship present.*

12. Which psychotherapy concept is most accurately described by the following: the client re-enacts a dream in the present and may speak as different people, objects or elements of the dream?

- A. Gestalt dream work**
- B. Gestalt therapy
- C. Interpretation
- D. Empty-chair technique

Answer: A - Gestalt dream work. Rather than imposing a fixed interpretation, Gestalt dream work invites the client to own and experience parts of the dream.

Memory aid: *Dream it, become it, speak it.*

13. Which psychotherapy concept is most accurately described by the following: turning back onto oneself what one wants to do to another, or doing to oneself what one wishes another would do?

- A. Cognitive behavioral therapy (CBT)
- B. Retroflexion**
- C. Empty-chair technique
- D. Gestalt therapy

Answer: B - Retroflexion. Retroflexion is a Gestalt contact-boundary disturbance directed back toward the self.

Memory aid: *RETRO = return action back to self.*

14. In a session with Joel, the counselor emphasizes an approach emphasizing freedom, responsibility, choice, meaning, authenticity, isolation and awareness of mortality. This BEST illustrates:

- A. Interpretation
- B. Gestalt dream work
- C. Dialectical Behavior Therapy (DBT)
- D. Existential therapy**

Answer: D - Existential therapy. Existential therapy helps clients face givens of existence and make authentic choices rather than simply trying to eliminate all anxiety.

Memory aid: *FMRC: Freedom, Meaning, Responsibility, Choice.*

15. In a session with Patricia, the counselor emphasizes learning by association between stimuli so that a previously neutral cue comes to elicit a response. This BEST illustrates:

- A. Operant conditioning
- B. Cognitive restructuring
- C. Flooding
- D. Classical conditioning**

Answer: D - Classical conditioning. Classical conditioning explains learned associations such as fear conditioning and underlies some exposure-based principles.

Memory aid: *Classical = cue-to-cue association.*

SECTION B: TRUE/FALSE - EXPLANATIONS

1. **TRUE.** The empty-chair technique is commonly associated with Gestalt therapy.

It is a classic Gestalt experiment for unfinished business or polarities.

2. FALSE. Existential therapy aims to eliminate every form of anxiety from life.

Existential therapy accepts that some anxiety is inherent in freedom, responsibility and existence; the aim is authentic engagement, not zero anxiety.

3. TRUE. Adlerian therapy emphasizes purposeful, goal-directed behavior and personal responsibility.

Those ideas are explicitly emphasized in the course audio opening.

4. TRUE. CBT assumes thoughts, emotions and behaviors can influence one another.

This reciprocal link is a core CBT formulation.

5. TRUE. DBT teaches mindfulness, distress tolerance, emotion regulation and interpersonal effectiveness.

These are the four major DBT skills domains.

SECTION C: MODEL SHORT ANSWERS

1. State two core conditions of person-centered therapy. (2mks)

- Any two: accurate empathy, unconditional positive regard, congruence/genuineness.

Memory aid: EUC.

2. Give the ABC elements of REBT. (2mks)

- A = Activating event

- B = Belief about the event

- C = emotional/behavioral Consequence

Memory aid: ABC: event does not mechanically create emotion; beliefs mediate.

3. Explain negative reinforcement with an example. (1mks)

- Negative reinforcement increases a behavior by removing an aversive stimulus.

- Example: fastening a seat belt to stop the warning sound makes future seat-belt fastening more likely.

Memory aid: Reinforcement increases; negative removes.

PART THREE: ETHICAL/LEGAL - ANSWERS

1. A counselor asks which principle or professional concept is defined by a departure from customary therapeutic boundaries that may be clinically appropriate and non-exploitative in context. The BEST answer is:

- A. Technology ethics
- B. Fidelity
- C. Boundary crossing**
- D. Autonomy

Answer: C - Boundary crossing. A boundary crossing is not automatically unethical; context, culture, intent and effect matter.

Memory aid: Crossing may be thoughtful; violation causes or risks serious harm/exploitation.

2. A counselor asks which principle or professional concept is defined by a serious harmful or exploitative breach of therapeutic boundaries. The BEST answer is:

- A. Boundary violation**
- B. Gift decision-making
- C. Professional competence
- D. Ethical response to colleague misconduct

Answer: A - Boundary violation. Boundary violations exploit the therapeutic relationship or cause significant harm and are unethical.

Memory aid: Violation = harm/exploitation.

3. During supervision, Nadine describes sexual or romantic involvement with a current therapy client. Which concept should guide the discussion?

- A. Justice
- B. Sexual relationship with a current client**
- C. Professional competence
- D. Dual or multiple relationship

Answer: B - Sexual relationship with a current client. Sexual relationships with current clients are prohibited because of power imbalance and exploitation risk. Terminating therapy to start a relationship does not erase those concerns.

Memory aid: Current client + sex = no.

4. During supervision, Fidel describes considering clinical meaning, value, timing, culture, power and exploitation risk before accepting or declining a client gift. Which concept should guide the discussion?

- A. Beneficence
- B. Gift decision-making**
- C. Malpractice
- D. Privileged communication

Answer: B - Gift decision-making. Gifts require contextual judgment. A culturally meaningful modest gift may be handled differently from an expensive or coercive gift.

Memory aid: VALUE: Value, Understanding/meaning, Local culture, Unequal power, Effect on therapy.

5. A counselor asks which principle or professional concept is defined by accepting goods or services instead of money when ethically permissible, not exploitative and not clinically contraindicated. The BEST answer is:

- A. Privacy
- B. Professional competence
- C. Bartering**
- D. Confidentiality

Answer: C - Bartering. Bartering may be acceptable under some codes when not exploitative and when arrangements are clear and clinically appropriate.

Memory aid: Barter only if Boundaries and Benefits are safe.

6. A counselor asks which principle or professional concept is defined by maintaining accurate, timely, secure and clinically appropriate client records. The BEST answer is:

- A. Privileged communication
- B. Record keeping**
- C. Duty to warn/protect
- D. Confidentiality

Answer: B - Record keeping. Records support continuity of care, accountability and legal/ethical duties; access must be controlled and retention rules followed.

Memory aid: ACTS: Accurate, Current, Timely, Secure.

7. A counselor asks which principle or professional concept is defined by ending therapy responsibly with preparation, continuity planning and referral when needed rather than abandoning the client. The BEST answer is:

- A. Professional competence
- B. Beneficence
- C. Termination and referral**
- D. Nonmaleficence

Answer: C - Termination and referral. Ethical termination includes adequate notice, discussion, referrals or continuity plans when appropriate.

Memory aid: PLAN: Prepare, Link/referral, Address feelings, Next steps.

8. During supervision, Patricia describes using ethical codes, consultation/supervision and appropriate reporting procedures when credible serious misconduct is observed. Which concept should guide the discussion?

- A. Ethical response to colleague misconduct**
- B. Privacy
- C. Privileged communication
- D. Fidelity

Answer: A - Ethical response to colleague misconduct. Serious ethical violations should not simply be ignored; follow code, organizational and legal procedures while protecting clients and due process.

Memory aid: C-A-R-E: Consult code, Assess facts, Report appropriately, Ensure client safety.

9. During supervision, Daniel describes protecting privacy, competence, informed consent, identity, emergency planning and data security when using digital or remote services. Which concept should guide the discussion?

- A. Technology ethics**
- B. Ethical response to colleague misconduct
- C. Nonmaleficence
- D. Informed consent

Answer: A - Technology ethics. Technology changes the medium, not the professional duty. Security, consent and contingency planning remain essential.

Memory aid: SPACE: Security, Privacy, Access/identity, Consent, Emergency plan.

10. A counselor asks which principle or professional concept is defined by not automatically disclosing the entire file on receipt of a subpoena; consult applicable law, the client/attorney and professional guidance, and disclose only as authorized or legally required. The BEST answer is:

- A. Fidelity
- B. Subpoena response**
- C. Privileged communication
- D. Boundary violation

Answer: B - Subpoena response. A subpoena and a court order are not always identical. The safest course-based principle is not to reflexively release all records; seek proper legal/ethical guidance and protect confidentiality to the extent permitted.

Memory aid: STOP: See legal guidance, Tell/consult client, Only required information, Protect confidentiality.

11. During supervision, Uma describes respecting the client's right to make informed choices about treatment. Which concept should guide the discussion?

- A. Subpoena response
- B. Autonomy**
- C. Bartering
- D. Privacy

Answer: B - Autonomy. Autonomy supports informed choice, consent and respect for the client's self-determination.

Memory aid: A = Allow informed choice.

12. During supervision, Owen describes acting to promote the client's welfare and benefit. Which concept should guide the discussion?

- A. Boundary violation
- B. Beneficence**
- C. Technology ethics
- D. Informed consent

Answer: B - Beneficence. Beneficence asks the professional to actively contribute to the client's well-being.

Memory aid: BENEFIT = beneficence.

13. A counselor asks which principle or professional concept is defined by avoiding or minimizing harm. The BEST answer is:

- A. Nonmaleficence**
- B. Privacy
- C. Record keeping
- D. Autonomy

Answer: A - Nonmaleficence. Nonmaleficence is the duty to do no harm and to avoid exposing clients to unnecessary risk.

Memory aid: NON-maleficence = NO harm.

14. In professional practice, a situation calls for fair and equitable distribution of services, benefits, burdens and opportunities. Which ethical/legal concept is MOST directly involved?

- A. Justice**
- B. Dual or multiple relationship
- C. Beneficence
- D. Gift decision-making

Answer: A - Justice. Justice concerns fairness and equitable treatment, including access and resource allocation.

Memory aid: Justice = fairness.

15. A counselor asks which principle or professional concept is defined by truthfulness and honest communication with clients. The BEST answer is:

- A. Subpoena response
- B. Ethical response to colleague misconduct
- C. Beneficence
- D. Veracity**

Answer: D - Veracity. Veracity requires accurate, truthful information rather than deception.

Memory aid: VERACITY = verifiable truth.

SECTION B: MODEL SHORT ANSWERS

1. State two technology-related ethical concerns in teletherapy. (2mks)

- Examples: data security/privacy, informed consent, identity verification, competence with technology, emergency location/plan, jurisdiction.

Memory aid: SPACE.

2. How should a therapist respond to a subpoena for records? (1mks)

- Do not reflexively release the entire file.

- Consult applicable law/professional guidance and the client/attorney; protect confidentiality and disclose only what is authorized or legally required.

Memory aid: STOP.

3. What is the principle of justice? (1mks)

- Fair and equitable treatment and distribution of benefits, burdens and resources.

Memory aid: Justice = fairness.

4. What does duty to warn/protect mean in therapy? (1mks)

- When a serious credible threat to an identifiable person is present, the therapist may have to take reasonable protective action according to applicable law and ethics, even though confidentiality is normally protected.

Memory aid: TIP: Threat, Identifiable person, Protect.

SECTION C: MODEL SHORT ESSAYS

1. Define mandated reporting. (2mks)

- A legal/professional requirement to report specified situations such as suspected abuse to designated authorities; it is a limit to confidentiality.

Memory aid: Mandatory = must report when the law says so.

2. State two ways a therapist should manage sexual or romantic feelings toward a client. (2mks)

- Acknowledge and monitor the feelings rather than act on them.

- Seek supervision/consultation and strengthen boundaries; refer if objectivity/competence cannot be maintained.

Memory aid: FEEL, do not ACT: notice, supervise, protect boundaries.

3. What should a therapist do when credible serious unethical behavior by a colleague is suspected? (1mks)

- Clarify facts, consult the code/supervisor as appropriate, protect the client and follow formal reporting procedures when required.

Memory aid: CARE: Consult, Assess, Report, Ensure safety.

PART FOUR: CULTURE / GROUP / COUNSELING PROCESS / TREATMENT PLAN - ANSWERS

1. A trainee observes using a qualified interpreter rather than relying on a child or untrained friend when language barriers affect care. This is an example of:

- A. Professional interpreter use**
- B. Therapeutic confrontation
- C. Support group
- D. Group role

Answer: A - Professional interpreter use. Qualified interpreters improve accuracy, confidentiality and informed consent.

Memory aid: *Interpreter = accuracy + confidentiality.*

2. In counseling practice, Fidel encounters a situation involving a group emphasizing mutual support, shared experience, acceptance and coping among people facing similar problems. Which concept BEST applies?

- A. Norming stage
- B. Adjourning stage
- C. Support group**
- D. Case formulation

Answer: C - Support group. Support groups reduce isolation and offer mutual aid through shared experience.

Memory aid: *SUPPORT = Similar people Understanding and Providing Peer help.*

3. In counseling practice, Amina encounters a situation involving a structured group focused on teaching information, understanding a condition and coping knowledge. Which concept BEST applies?

- A. Professional interpreter use
- B. Skills-development group
- C. Psychoeducational group**
- D. Goal setting

Answer: C - Psychoeducational group. Psychoeducational groups primarily teach information and understanding; they may include discussion and practice.

Memory aid: *EDU = education is central.*

4. Which counseling/group/cultural concept is BEST described by: a group focused on learning and practicing specific behavioral or interpersonal skills?

- A. Psychoeducational group
- B. Norming stage
- C. Skills-development group**
- D. Adjourning stage

Answer: C - Skills-development group. Skills groups emphasize rehearsal, practice and feedback rather than information alone.

Memory aid: *SKILLS = Show, Keep practicing, Improve, Learn, feedback.*

5. A trainee observes a structured group using cognitive and behavioral methods such as thought monitoring, skills practice and behavioral exercises. This is an example of:

- A. Cognitive-behavioral group**
- B. Termination and follow-up
- C. Self-disclosure
- D. Therapeutic confrontation

Answer: A - Cognitive-behavioral group. CBT groups combine structured learning with practice and feedback around thoughts and behaviors.

Memory aid: *Group CBT = Think + Do together.*

6. In counseling practice, Hassan encounters a situation involving a recurrent functional position adopted by a member, such as scapegoat, monopolizer, peacemaker or spokesperson. Which concept BEST applies?

A. Group role

- B. Cognitive-behavioral group
- C. Reflection of feeling
- D. Psychoeducational group

Answer: A - Group role. Group roles describe patterned ways members participate and influence group dynamics.

Memory aid: Role = repeated position in the group.

7. Which counseling/group/cultural concept is BEST described by: an agreement that members protect information disclosed by other members, while the leader explains that peer confidentiality cannot be guaranteed in the same way as professional confidentiality?

- A. Progress review
- B. Termination and follow-up
- C. Reflection of feeling

D. Group confidentiality

Answer: D - Group confidentiality. Confidentiality is essential in groups, but leaders should explain realistic limits because members are not all licensed professionals.

Memory aid: What is said in group should stay in group - with limits explained.

8. A trainee observes early group stage characterized by orientation, uncertainty, dependence and testing. This is an example of:

- A. Forming stage**
- B. Norming stage
- C. Paraphrasing
- D. Assessment

Answer: A - Forming stage. Forming is the beginning stage in which members learn the purpose, structure and safety of the group.

Memory aid: FSNPA: Forming, Storming, Norming, Performing, Adjourning.

9. A trainee observes group stage in which conflict, resistance, competition or emotional testing becomes more visible. This is an example of:

- A. Therapeutic confrontation
- B. Cultural competence
- C. Storming stage**
- D. Professional interpreter use

Answer: C - Storming stage. Storming often brings disagreement and challenges to roles or leadership.

Memory aid: Storm = conflict weather.

10. Which counseling/group/cultural concept is BEST described by: group stage in which cohesion, trust, norms and cooperation consolidate?

- A. Paraphrasing
- B. Norming stage**
- C. Self-disclosure
- D. Intervention stage

Answer: B - Norming stage. Norming is marked by clearer expectations and stronger cohesion.

Memory aid: Norming = norms settle.

11. Which counseling/group/cultural concept is BEST described by: mature working stage in which members use the group productively toward goals?

- A. Norming stage
- B. Cultural competence
- C. Performing stage**
- D. Adjourning stage

Answer: C - Performing stage. Performing is the task-focused, therapeutically productive phase.

Memory aid: *Performing = productive work.*

12. Which counseling/group/cultural concept is BEST described by: ending stage involving review, separation, consolidation and transfer of learning?

- A. Adjourning stage**
- B. Psychoeducational group
- C. Treatment plan
- D. Support group

Answer: A - Adjourning stage. Adjourning addresses termination and helps members carry gains beyond the group.

Memory aid: *Adjourning = goodbye and generalize gains.*

13. Which counseling/group/cultural concept is BEST described by: creating safety, trust, empathy and collaborative engagement at the beginning of counseling?

- A. Self-disclosure
- B. Ethnocentrism
- C. Paraphrasing
- D. Rapport building**

Answer: D - Rapport building. Rapport supports disclosure and collaboration; it begins early but continues throughout treatment.

Memory aid: *RAPPORT starts with Respect, Attention, Presence.*

14. Which counseling/group/cultural concept is BEST described by: systematically exploring the client's problems, symptoms, context, history, strengths, risk and functioning?

- A. Assessment**
- B. Performing stage
- C. Group role
- D. Paraphrasing

Answer: A - Assessment. Assessment precedes or accompanies formulation and goal setting; it includes both problems and resources.

Memory aid: *HRSF: History, Risk, Symptoms, Functioning.*

15. Which counseling/group/cultural concept is BEST described by: collaboratively defining client-centered outcomes that are realistic and observable?

- A. Professional interpreter use
- B. Therapeutic confrontation
- C. Goal setting**
- D. Norming stage

Answer: C - Goal setting. Goals are stronger when the client participates and the goals are specific enough to evaluate.

Memory aid: *SMART: Specific, Measurable, Achievable, Realistic/Relevant, Time-bound.*

SECTION B: MODEL SHORT ESSAYS

1. Differentiate paraphrasing from reflection of feeling. **(4mks)**

- Paraphrasing restates the content/meaning in new words.
- Reflection of feeling identifies and feeds back the emotion or emotional meaning.

Memory aid: *Paraphrase = facts/meaning; reflection = feeling.*

2. Why should a qualified interpreter be preferred to a child family member? **(4mks)**

- Improves accuracy and reduces role burden on the child.
- Better supports confidentiality, informed consent and sensitive disclosure.

Memory aid: *Accuracy + confidentiality + appropriate role.*

3. State four tasks of ethical termination. **(2mks)**

- Review progress and goals.
- Discuss feelings and reasons for ending.
- Consolidate skills/relapse prevention.
- Arrange follow-up/referral/continuity when needed.

Memory aid: *RECAP*.

SUPERVISED VOCATIONAL QUALIFICATION DIPLOMA - EXAMINATION

Session: Revision Mock Set 07	Duration: 1hr 30mins
Specialty: Psychotherapy	Coefficient: 04
Eliminatory mark: < 14/20	Training reference: VTI-AEDA examination style

SIMULATION OF REAL WORKING SITUATION PAPER - SOLUTION / MARKING GUIDE PRACTICE SET 07

Revision material based on the supplied VTI-AEDA 2025 examination format and the supplied psychotherapy course recordings. Not an official examination paper.

DETAILED MODEL ANSWERS - SIMULATION OF REAL WORKING SITUATIONS

SECTION I

1. Dependence on authority and Adlerian formulation - anchor: Adlerian therapy

Ruth, 55, says she has always felt "less capable than everyone else" and avoids decisions unless a supervisor or parent approves them. she wants to become more independent but expects failure before trying.

Model response / marking points (25 marks):

1. Establish an egalitarian, encouraging relationship.
2. Explore early experiences, family context and the client's private logic/lifestyle conclusions.
3. Identify inferiority feelings and the goal served by avoiding decisions (e.g., avoiding failure/disapproval).
4. Emphasize behavior as purposeful rather than simply caused by the past.
5. Explore social relatedness and belonging.
6. Use encouragement to separate mistakes from global worth.
7. Reframe responsibility as the capacity to choose new action.
8. Set a small "act as if" or graded independent decision experiment.
9. Review consequences and revise mistaken beliefs/goals.
10. Maintain collaboration rather than therapist authority, so therapy does not reproduce dependency.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: GSCR: Goal-directed, Social relatedness, Conscious choice, Responsibility.

2. Early relationships and psychoanalytic formulation - anchor: Psychoanalytic/psychodynamic therapy

Simon, 62, repeatedly chooses distant partners and becomes intensely angry when the therapist reschedules a session, saying, "People always leave when I need them." Childhood involved repeated separation from a primary caregiver.

Model response / marking points (25 marks):

1. Develop a therapeutic alliance and invite free exploration of current and early relationships.
2. Notice the recurring abandonment theme.
3. Recognize possible transference when the therapist's rescheduling is experienced through earlier expectations of abandonment.
4. Attend to resistance/avoidance without shaming.
5. Use clarification before deeper interpretation.
6. Offer tentative, timed interpretations rather than declaring hidden truth.
7. Monitor countertransference in the therapist and use supervision.
8. Connect past pattern, present relationships and here-and-now therapy when the client can use the insight.

9. Maintain boundaries and consistency.
10. Help the client develop insight and new relational choices.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: *FIRT: Free association, Interpretation, Resistance, Transference.*

SECTION II

1. Counseling-skills micro-session - anchor: Core counseling skills

Theresa, 21, says, "Everything at home is falling apart. I am angry all the time, but when anyone asks what is wrong I just say I am fine." During the session Theresa speaks quickly, looks down and repeatedly clenches both hands.

Model response / marking points (25 marks):

1. Use attending behavior and silence to communicate presence.
2. Paraphrase the content without parroting.
3. Reflect emotion: anger, distress, perhaps fear or helplessness, checking accuracy.
4. Use open questions such as "What happens at home when the anger builds?"
5. Notice nonverbal cues.
6. Use gentle confrontation of discrepancy: saying "fine" while describing collapse/clenched hands.
7. Avoid judgment or advice before understanding.
8. Summarize themes and check accuracy.
9. Assess safety if statements suggest hopelessness/violence/self-harm.
10. Collaboratively identify what the client wants from counseling.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the vignette.

Memory aid: *LEAPS: Listen, Empathize, Ask, Paraphrase, Summarize.*

2. Work stress and first counseling session - anchor: First-session counseling process with person-centered engagement plus problem-focused assessment

Valerie, 28, works as a mechanic and has recently been working 11-12 hour days. he has become irritable with colleagues, sleeps poorly, has stopped seeing friends and says, "I am losing control of my life." Valerie has never attended counseling before and is unsure what to expect.

Model response / marking points (25 marks):

1. Establish rapport, privacy and a respectful therapeutic setting.
2. Explain role, informed consent and limits of confidentiality.
3. Use open questions, active listening, paraphrase, reflection and summarizing.
4. Assess stressors, sleep, functioning, substance use, supports, medical/mental-health history and risk including suicide/self-harm.
5. Identify strengths and the client's own priorities.
6. Agree on a preliminary formulation: overload, reduced recovery, irritability and impaired balance; avoid premature diagnosis.
7. Set one or two SMART initial goals.
8. Introduce an appropriate first coping intervention such as breathing/relaxation, activity scheduling, boundaries or problem solving.
9. Agree on homework/self-monitoring and next appointment.
10. Close by summarizing, checking understanding and inviting questions.

Suggested mark allocation: Award marks for correct formulation/approach, risk and ethical priorities, logical session structure, appropriate technique application, collaboration/homework/follow-up, and professional communication. Credit clinically sound alternatives when clearly justified. Do not award full marks for naming a theory without showing how it is applied to the facts of the

vignette.

Memory aid: *RAPPORT*: Rapport, Agreement/consent, Problem assessment, Prioritize risk, Objectives, Response/technique, Take-home/follow-up.